VILDAGLIPTIN

Check LFTs
Twice daily

CHEMICAL NAME (2S)-1-{2-[(3-hydroxyadamantan-1-yl)amino]acetyl}pyrrolidine-2-carbonitrile
CLASS Dipeptidyl peptidase-4 (DPP-4) inhibitor
TRADE NAMES Galvus · Galvus Met (with metformin) · Jalra · Zomelis

THERAPEUTIC USES / INDICATIONS

- Type 2 diabetes; monotherapy (metformin intolerance) or dual/triple therapy with metformin, sulfonylurea, insulin.

PHARMACODYNAMICS

- Inhibits **DPP-4** → ↑ GLP-1 and GIP → glucose-dependent insulin secretion; ↓ glucagon.

ADMINISTRATION

- **PO** with or without food; twice daily (once daily with sulfonylurea).

DOSAGES

- **Adult:** 50 mg twice daily (morning and evening); max 100 mg/day.
- **With sulfonylurea:** 50 mg once daily in the morning.
- **Moderate–severe renal impairment:** 50 mg once daily.

PHARMACOKINETICS

- **Onset:** 1–2 h. **Duration:** ~12–24 h.
- Hydrolysed (not CYP) to inactive metabolite.
- Excreted in urine. **t½:** 2–3 h (DPP-4 inhibition lasts longer).

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Dizziness, headache.
- Tremor (with metformin).
- Peripheral oedema.

ADVERSE EFFECTS (SERIOUS)

- Hepatic dysfunction/hepatitis (rare).
- Pancreatitis.
- Bullous pemphigoid; angioedema.

CONTRAINDICATIONS & CAUTIONS

- Hepatic impairment (incl. ALT/AST >3× ULN).
- Type 1 diabetes, DKA.
- Hypersensitivity.

KEY INTERACTIONS

- **Sulfonylureas, insulin:** hypoglycaemia (reduce their dose).
- **ACE inhibitors:** ↑ angioedema.

NURSING MANAGEMENT

- LFTs before starting, every 3 months for the first year, then periodically.
- Monitor HbA1c, renal function.
- Watch for jaundice, abdominal pain.

PATIENT EDUCATION

- Take twice daily (or once with sulfonylurea).
- Report yellow skin, dark urine, severe stomach pain, skin blisters.
- Hypoglycaemia advice if combined with sulfonylurea/insulin.

KEY POINTS

- DPP-4 inhibitor, 50 mg BD.
- Weight-neutral; low hypo risk alone.
- LFT monitoring required.
- Pancreatitis, pemphigoid.